



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متدد

إسم المريض ربيعاً : صفيه ناصر علي عباس
المحافظة : القاهرة الحي / المدينة : مدينه نصر الاول العنوان : الجنائني
رقم البطاقة : 29802172401102 رقم التليفون : 01115323617 تاريخ تسجيل الإستمارة : 04-10-2026
العمر (تاريخ الميلاد) : 17-02-1998 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج بمشبطات [13519] CDK4/6 قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ----- التوقيع : -----
الاسم : د/ندى حسن الوظيفة : ----- التوقيع : -----
الاسم : د/نرمين رمضان الوظيفة : ----- التوقيع : -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة الأبحاث والمرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: صفية ناصر علي عباس
National ID: 29802172401102



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي CDK4/6 inhibitor therapy

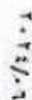
الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



33A-71V P 2-11-6

ریبہ منزل

1

5.

المطابقة سارية حتى ٢٠٣١/٠٦/٢



بطاقة كاتوليكية

صفحة

ناصر علی عباس

اش - للناظر صلاح الدين محمد بن الجليلي

مدينة نصر اول - القاهرة

1994/11



78A-71V4E-99-4

KP1545707

04/20/2026

MDT FORM

Name:	Safya Naser Ali Huseen	MRN:	5145
Age:	28 Y	ID:	29802172401102
Menopausal Sta	pre	Phone:	01044875760
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC
ER: 8/8 PR: 5/8 HER2: +1 KI67: 20%

R

L

Sonomammography:Mets Workup:History:

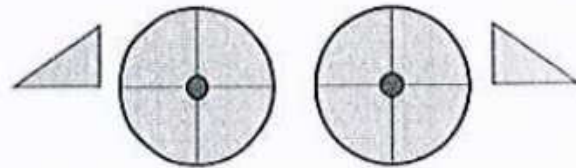
28 YEARS OLD FEMAR PT WITH MBC TO BONE+LN ON
tamoxifen+ZOLADEX+zomata since 9/2024 , IDC ER 8/8 PR 5/8 HER2+1 KI67
20%
bone scan 4/2026 : DP (with subjective progression)

Recommendations:

2nd line CDK4/6 inhibitor + femara .

MDT Outcome & Plan

CDK4/6 inhibitor + femara +ZOLADEX



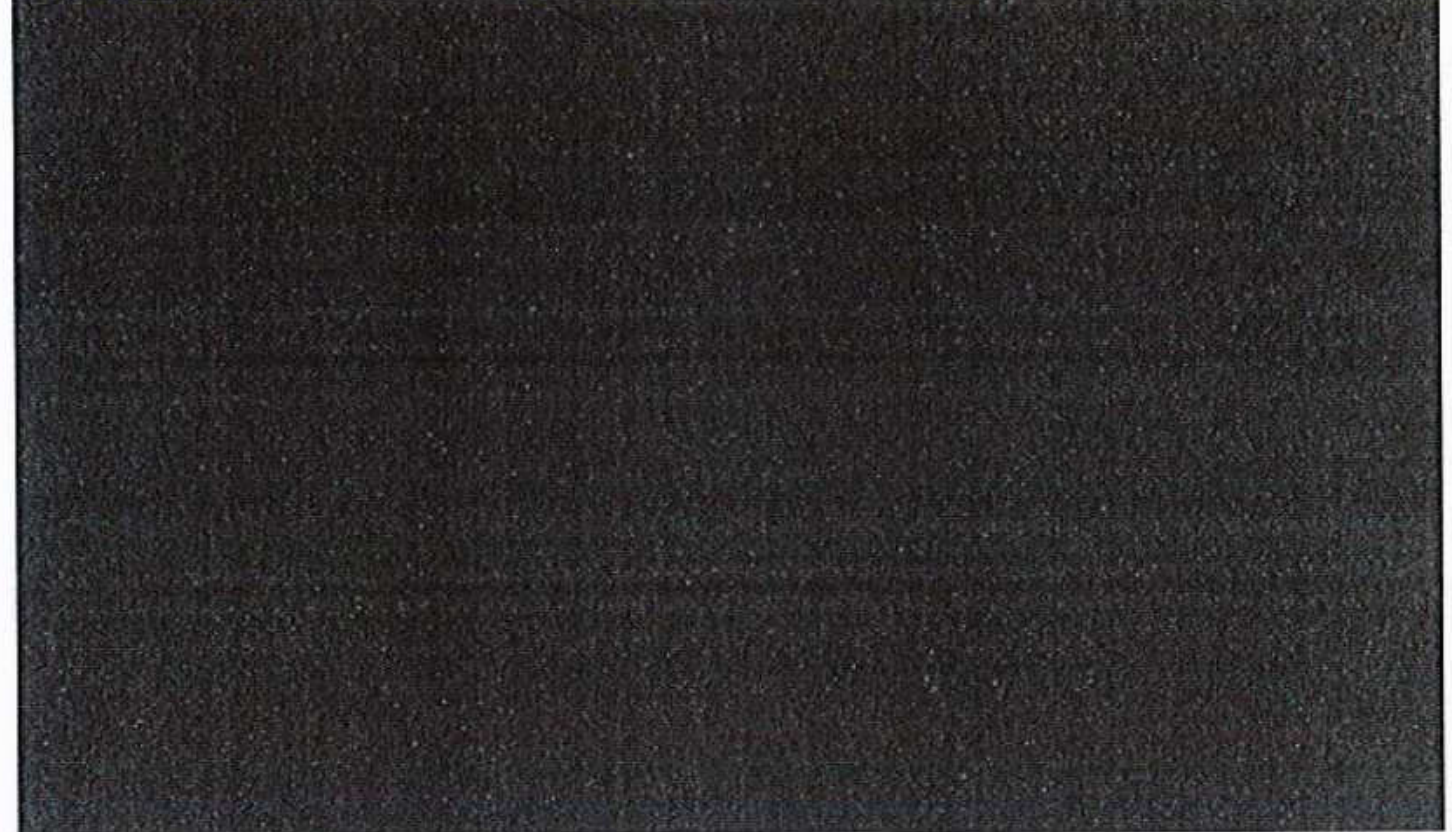
T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ ارمين مصطفى					

C. ٤٧٤/٢٠

Safya Naser Ali Huseen



Name: صفيه ناصر علي عباس		GUSTAVE ROUSSY	
MR. : 5145			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	
ننشر بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 28-year-old female with metastatic breast cancer to bone and lymph nodes, diagnosed with invasive ductal carcinoma ER 8/8, PR 5/8, HER2+1, Ki-67 20%. Initial imaging on 24/01/2026 showed a 19 × 17 mm right breast lesion, a 22 × 11 mm right axillary lymph node, and 16 × 8 mm right pulmonary nodules. A bone scan was performed and Zometa 4 mg was initiated with plans for Zoladex 10.8 mg and Femara 1 mg daily, along with calcium supplementation. By 12/04/2026 the patient had been on tamoxifen, Zoladex, and Zometa since 09/2024. Bone scan in 04/2026 demonstrated disease progression. A multidisciplinary team recommended second-line therapy with a CDK4/6 inhibitor plus Femara, and a pain consultation was arranged. On 12/07/2026 the patient was seen for continuation of Femara and Zoladex 10.8 mg, and was started on cycle 2 of Ibrance 125 mg. Laboratory values were within acceptable limits. On 09/08/2026 the patient reported vomiting during the first few days of starting Ibrance. CBC was acceptable; AST was 51 U/L, ALT 61 U/L, and serum calcium was elevated at 10.10 mg/dL. Management included: (1) continuation of cycle 3 of Femara/Ibrance with Zoladex scheduled for October; (2) discontinuation of calcium supplements and repeat serum calcium in one month; (3) continuation of Zometa every 3 months; and (4) CT chest and bone scan follow-up next month. On 31/08/2026 the MDT confirmed the regimen of a CDK4/6 inhibitor plus Femara and Zoladex, with Zometa q 3 months. The patient is on cycle 3 of Ibrance and Femara, with the next visit scheduled for 28/09/2026. The patient has completed 3 cycles of Ibrance.			
تحريراً في ,			
التاريخ : 21-09-2026		الوقت :	
الطبيب المعالج :		شيماء فاروق يحيى لاشين	
يعتمد ,			



الوزارة العامة للصحة
وزارة الصحة والبيئة



8 Patient ID
1 Name

Serial number
83225209
Old Med:

83225209
Old Med:

2667-2024

Initial Age: 27 Y



Primary history

Current status

Comorbidity

MR

MR

MR

BM1

GROUP

Initial STAGE

REASON OF MDT

Date of MDT

08/09/2024

Mets Workup:

pet ct: enlarged hypermetabolic mediastinal, hilar, pt
axillary subpectoral LN with supra retroclavicular LN
with few hyper metabolic osseous lesions

Her 2 Neu

SISH

K157
21

M
LNs

enlarged hypermetabolic looking 6
axillary LN

pet ct

enlarged hypermetabolic mediastinal, hilar, pt

axillary subpectoral LN with supra retroclavicular LN

with few hyper metabolic osseous lesions

Pantoxifen for 2 days then start to 41 after E2 LEVEL

For, NEMDT

MEDICAL ONCOLOGY

SURGERY

Pathology

RADIOLOGY

PATHOLOGY

Nutrition

(بصحة / شفاء)



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذة / صفية ناصر علي عباس	Request No.	: 13005524
Patient No.	: 29802172401102	Request Date	: 23/07/2024
Path No.	: 2553/24	Verification Date	: 05/08/2024
Sex	: Female	Financial Category	: ارام دار السلام - نفقة الدولة
Age	: 26 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical data: Right breast malignant looking mass with pathological lymph node.

Nature of Specimen: Tru-cut biopsy, collected at 22/7/2024, 11:40 am.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 12% of tumor cells (score 5/8).
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).
- Ki67: 22% nuclear reaction of tumor cells.

Conclusion:

Right breast malignant looking mass with pathological lymph node, Tru-cut biopsy: invasive duct carcinoma, Grade II, with focal DCIS and lymphovascular invasion.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: score 1+.
- Ki67: Positive in 22% of tumor cells.

Scoring system: "ER, PR"

Verified by :

DR.Menna Nabil

Printing Date : 27/08/2024 12:43:29

Page 1 of 2

Printed By : Mrs.Mayada Hegazy





Patient Name الأستاذة / صفية ناصر على عباس
Patient No. 29802172401102
Path No. 2553/24
Sex Female
Age 26 Y
Referreing Doc.

Request No. 13005524
Request Date 23/07/2024
Verification Date 05/08/2024
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient

Registered by Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, $\leq 1\%$; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $< 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $< 10\%$ of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within $< 10\%$ of the invasive tumor cells or complete and circumferential membrane staining that is intense and within $< 10\%$ of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense $< 10\%$ of tumor cells.

Comment: Although Her2 is Negative, such cases (Score 1+) are predictive of response to Her2-Ab drug conjugates (ADCs) according to latest 2023 ASCO-CAP guidelines.

Signature

Prof. Dr. Lubna Omar.

Dr. Menna Nabil.

Verified by :

DR.Menna Nabil

Printing Date : 27/08/2024 12:43:29

Page 2 of 2

Printed By : Mrs.Mayada Hegazy





Patient Name : الأستاذة / صفية ناصر على عباس	Request No. : 13005524
Patient No. : 29802172401102	Request Date : 23/07/2024
Path No. : 2553/24	Verification Date : 30/07/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 26 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data: Right breast malignant looking mass with pathological lymph node.

Nature of Specimen: Tru-cut biopsy, collected at 22/7/2024, 11:40 am.

Gross Examination:

- Multiple fragmented grayish white tissue cores up to 0.5 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focally positive.
- * Perineural invasion: Negative.
- * Angioinvasion: Positive.

Diagnosis:

Right breast malignant looking mass with pathological lymph node, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS and lymphovascular invasion.
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Toka Medhat.

Verified by :
DR.Menna Nabil

Printing Date : 27/08/2024 12:43:44

Page 1 of 1

Printing By : Mrs.Mayada Hegazy



١٠٤٣ من كورنيليس النيل . قم الخليج . القاهرة

Patient Name: Safeya Nasser Ali Abbas	Date : 25/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 27 Year(s)
Medical ID: 2667/2024	Pat. ID: 77528

FOLLOWUP POST-CONTRAST MULTIPLE SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS REVEAL:

- Known case of cancer right breast
- Compared to the last study dated 26/02/2025

I. Chest:

- **Stationary course of:**
 - Right breast heterogenous parenchyma with overlying skin thickening as well as the small nodule seen at its inferior aspect with no significant changes since last study? nodal? focal lesion.
 - Left axillary lymph nodes largest measuring about 1.6 cm
 - Resolution of the bilateral interstitial and centrilobular thickening.
 - Stationary course of the pleural nodules.
 - Resolution of left breast nodule.
 - Minimal regression of the size of the multiple pathologically enlarged right axillary lymph nodes noted at level I, II and III, largest measuring 18x 11 mm compared to 18 x 14 mm.
 - Regression of the mild enlarged prevascular, pretracheal and precarinal lymph nodes with the largest seen at pretracheal group measuring about 1.8 cm compared to 2.2 cm.
 - No pleural or pericardial effusion.
 - No pulmonary parenchymal consolidation, cavitation or collapse
 - Normal non enhanced CT appearance of the heart and major thoracic vessels.

II. Abdomen & pelvis:

- The liver is mildly enlarged in size with homogenous low CT parenchymal attenuation and regular outline. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Patent homogeneously enhancing portal vein and its main branches.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal CT appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

• **Scanned skeleton:**

- D12 lytic focal lesion is still seen.
- Left femoral head sclerotic foci likely bone island.

OPINION:

Status follow up showing the above described changes.

Name : Safia Naser Ali Abbas

Date : 04-04-2026 11:30

Branch : Dokki

Medical No. : 342017

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

Tc-99m MDP SKELETAL SCINTIGRAPHY

- **CLINICAL HISTORY:** 28-year-old female patient with history of right breast cancer with bone metastasis on hormonal treatment complains of bony pain; referred for metastatic survey.
- **Technique:** Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

FINDINGS

Compared to the prior stud dated 20/09/2025,

- The previously reported lesion at the left greater trochanter is still noted.
- Newly detected osseous lesions are seen at occipital region of skull, anterior end of bilateral lower ribs, mid segment of left 7th rib posteriorly as well as costovertebral junction of right 7th rib.
- Osteoarthritic changes around big joints.

IMPRESSION:

- Persistent left greater trochanter lesion as well as new osseous lesion identified as above-detailed. Findings are impressive of progressive metastatic bone disease, for radiological correlation and ongoing oncologic follow-up.

MOHAMED GAMAL HAMED

Dr. Alaa Nagy

Dr. Alaa Nagy

Name : SARIA NASER ALI

Date : 24-01-2026 01:16

Branch : Portable

Medical No. : GRI-78528

Ref Physician Prof. CT CAP: C

Ins. Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of CA breast with

As compared to previous study dated 25/8/2025, the current study revealed

II. CHEST:

- Still seen right breast heterogenous parenchyma with overlying skin thickening with no significant changes of the small nodule
- Stationary course of the enlarged right axillary pathological lymph nodes, the largest is still measuring 1.8x1.1cm
- Stationary course of the enlarged mediastinal lymph nodes, the largest is seen at pre-tracheal group measuring 1.8cm
- Still seen left axillary lymph nodes, the largest is measuring 1.6cm
- Stationary course of the pleural and fissural nodules
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

III. ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous fatty parenchymal attenuation. No focal lesions
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

III examined skeleton:

- Still seen D12 lytic focal lesion
- Left femoral head sclerotic focus likely bone island

Opinion:

- Overall stationary course.

DR/NERMEEN RAMADAN.MD

Name : SAFYA NASER ALI

Date : 05-03-2026 02:28

Branch : Portable

Medical No. : GRI-050320269

Ref.Physician Prof :

Ins.Company :

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: pathologically proven right breast cancer, on hormonal therapy, follow up study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- UOQ posterior third speculated irregular high density mass lesion with anteriorly related rather linear densities seen reaching the nipple along with LOQ, posterior third similar masses. Accompanying architectural distortion as well as clusters of suspicious microcalcifications are noted.
- Thickened skin and coarsened trabeculae are noted.
- **Ultrasound** demonstrated outer breast multiple irregular speculated solid mass lesions, largest two are seen at posterior thirds of 10 o'clock, 12 mm deep to the skin measuring 10 x 16 mm and 6 o'clock, 2 mm deep to the skin measuring 16 x 10 mm seen resting on the underline muscle.
- Pathological ducts are seen at the upper quadrant reaching 3 mm with echogenic content reaching the nipple with multiple smaller satellites are seen scattered at the outer breast.
- Thickened skin reaching 4.5 mm and coarsened trabeculae.
- Normal skin/nipple/areola complex.
- Multiple (more than three) level 1 pathological axillary LNs are noted, two are seen matted, multiple pathological nodes are noted at levels 2, 3, and supra-clavicular as well.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with mottled appearance yet no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): outer breast multicentric process with multiple pathological axillary and supra-clavicular nodes; progressive disease.

Left breast (BIRADS 1): Normal study.

Recommendation:-

For correlation with CESM results.

Best Regards,
DR. Amal Hegazi, M.Sc.

١٩٩٨/٧
 KP1545707

٧٩٨٠٧١٧٢٠١١٠٧

الجمهورية الجزائرية الديمقراطية الشعبية
 وزارة الداخلية
 مديرية أمن ورقلة - ورقلة

الاسم: علي حسان
 المسمى: ضابط



٧٠٧٢/٩ ٧٩٨٠٧١٧٢٠١١٠٧

المنطقة: ورقلة

٧٠٧١/٧٢

المنطقة: ورقلة




إسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،،

بشأن تقرير طبي للمريض: *Ayad*

إيماء الطالب المقدم من

بقسم: رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

Breast Cancer

met.

Zaneta 4mg / 3m

.....

تحريراً في

التاريخ: *٢١ / ٨ / ٢٠٢٢*

الوقت:

الطبيب المعالج

Dr. Youssef

يُختم ،،،

إسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/.....

تحية طيبة وبعد،،،

إيماء للطلب المقدم من بشأن تقرير طبي للمريض:
بقسم رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

met breast cancer
for

Salader 10.8

.....

تحريراً في

التاريخ: / / الوقت:

الطبيب المعالج

.....

يؤكد ،،

إسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،،

بشأن تقرير طبي للمريض: *Ayad*

إيماء الطالب المقدم من

بقسم: رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

Breast Cancer

met.

Zometax 4mg / 3m

.....

تحريراً في

التاريخ: *٢١ / ٨ / ٢٠٢٢*

الوقت:

الطبيب المعالج

Dr. Youssef

يُختم ،،،

إسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/.....

تحية طيبة وبعد،،،

إيماء للطلب المقدم من بشأن تقرير طبي للمريض:
بقسم رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

met breast cancer
for

Salader 10.8

.....

تحريراً في

التاريخ: / / الوقت:

الطبيب المعالج

.....

يؤكد ،،

إسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،،

بشأن تقرير طبي للمريض: *Ayad*

إيماء الطالب المقدم من

بقسم: رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

Breast Cancer

met.

Zaneta 4mg / 3m

.....

تحريراً في

التاريخ: *٢١ / ٨ / ٢٠٢٢*

الوقت:

الطبيب المعالج

Dr. Youssef

يُختم ،،،

إسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/.....

تحية طيبة وبعد،،،

إيماء للطلب المقدم من بشأن تقرير طبي للمريض:.....

بقسم رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

met breast cancer
for

Salader 10.8

.....

تحريراً في

التاريخ: / / الوقت:

الطبيب المعالج

.....

يؤكد ،،،

Patient's Name: Safia Naser Ali Abbas
Date: 2026-08-31
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

OSSEOUS STRUCTURES

- **Small lytic lesion involving the T12 vertebral body**, which remains visualized compared with the prior examination. Its metastatic nature cannot be excluded in this oncologic setting.
- No new suspicious osseous lesion identified.

IMPRESSION:

1. **Slight interval increase in size of the right axillary lymph node**, measuring **25 × 13 mm**, suspicious for persistent nodal metastatic involvement.
2. **New faint right upper-lobe ground-glass opacity**, nonspecific, favoring inflammatory/infectious change; attention on follow-up is advised.
3. **Complete regression of the previously demonstrated pulmonary nodule**.
4. **Fatty hepatomegaly**, without definite focal hepatic metastasis.
5. **Persistent small lytic lesion at T12**; osseous metastatic disease cannot be excluded. Consider correlation with PET/CT or bone scan if clinically indicated.

Ibrahim



Name : Safia Naser Ali Abbas

Date : 04-09-2026 03:29

Branch : Dokki

Medical No.: 454265

Ref.Physician Prof : Goustav Roussey Egypt

Tc^{99m} MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

28 year-old female patient, with history of metastatic right breast cancer to the bone, referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

Considering the previous bone scan report dated 4/4/2026, the current study revealed regression of the metastatic bone disease evidenced by:

- Near complete disappearance of the previously depicted lesions at occipital region of the skull, anterior end of bilateral lower ribs, mid segment of left 7th rib posteriorly and costo-vertebral junction of right 7th rib .
- Marked regression in term of size and tracer uptake intensity is also noted at the other lesion involving the greater trochanter of left femur.
- No newly developed lesions could be elicited on the current study.
- Degenerative changes were also noted at different levels of the spine and around big joints.

OPINION

- Remarkable regression of the previously noted bone lesions as described above with no newly developed ones as described above indicating good response to the given therapy.

**Much Obligated
Dr. Alaa Nagy, MD
Nuclear medicine**